

SPLEEN — Clinical Revision Table

Bold = high-yield. Dr Dara Mohammed.

ANATOMY & FUNCTION	
Item	Key points
Anatomy	75–250g; LUQ along 10th rib . Gastrosplenic lig (ant, short gastric vessels); splenorenal lig (post, splenic vessels + pancreatic tail). Splenic vein + SMV → portal vein
5 functions	1. Immune (IgM, opsonins, T/B cells) 2. Filter (effete RBC/bacteria, recycle iron) 3. Pitting (removes Howell-Jolly/Heinz/target) 4. Reservoir (8% RBC) 5. Cytopoiesis
Investigation	US (size/cyst); CT contrast (main); MRI T2 (cysts); plain film calcification (aneurysm/old infarct/hydatid/TB)
CONGENITAL & VASCULAR	
Condition	Key points
Splenunculi (accessory)	10–30% , 50% near hilum; must remove at splenectomy or disease persists
Agenesis / polysplenia	Agenesis rare (5% congenital heart disease); polysplenia = failed fusion
Cysts	True (epithelial: dermoid/epidermoid) vs pseudocyst (no lining, common after pancreatitis)
Splenic artery aneurysm	Women 2× , main trunk, asymptomatic until rupture; ¼ ruptures in pregnancy (3rd trimester) ; Tx embolisation/stent
Splenic infarction	Massive spleen/sickle/vein thrombosis; LUQ + left shoulder tip pain ; conservative; splenectomy if septic abscess
TRAUMA & SPLENOMEGALY	
Topic	Key points
Splenic rupture	Most commonly injured intra-abdominal organ ; blunt LUQ trauma. CT (stable) , graded I–V. Resuscitate; splenectomy for severe; angio-embolisation; laparotomy if unstable (don't delay)
Splenomegaly causes	Infective (malaria, EBV, kala-azar, TB), blood (leukaemia, thalassemia, spherocytosis, sickle, AIHA), metabolic (Gaucher's , amyloid), circulatory (portal HTN), collagen (Felty's), neoplastic
Hypersplenism	Splenomegaly + cytopenia + marrow hyperplasia + improves after splenectomy (enlarges 3× before palpable)
Neoplasms	Haemangioma = commonest benign ; lymphoma = commonest neoplastic enlargement ; mets rare; myelofibrosis
SPLENECTOMY	
Aspect	Key points
Indications	Trauma (accidental/iatrogenic); oncological (en bloc); haematological (spherocytosis, ITP, hypersplenism) ; portal HTN
Preop	Correct coagulation (FFP/platelets); VACCINATE: pneumococcus + meningococcus C (q5yr), Hib (q10yr)
Complications	Haemorrhage (slipped ligature); left basal atelectasis + pleural effusion ; pancreatic tail injury (pancreatitis/fistula); thrombocytosis ; OPSI (overwhelming post-splenectomy infection)